

Meeting Proceedings Memo

Meeting Details

A focused group meeting was conducted with ICU faculty to comprehend the prognostication processes used by the intensivists. The session involved detailed discussion amongst physician stakeholders

Participants:

Intensivists with > 5 years of experience.

Dr. Bhuvana (HOD Dept of Critical care), Dr. Divya, Dr. Amarja, Dr. Carol, Dr. Vikram, Dr Deepali, Dr. Natesh

Dr Moses (HOD SICU), Dr Mathangi, Dr. Natasha

ICMR Research team: Ms Ann, Ms Risha and Dr Yamuna.

Dr Nandini, Head of Department, Pain and Palliative Medicine moderated the meeting.

Purpose of the Meeting

The meeting was conducted to

- explore and build consensus on prognostication processes and metrics
- build a practical framework to facilitate timely shared decisions, for seamless application irrespective of the experience of the intensivist

Aims

- timely involvement of palliative care services for appropriate patients
- facilitate withdrawal / deescalation/ withholding aggressive interventions where appropriate
- enable timely transitions in goals of care in concurrence with the family

Background

The enabling palliative care services occurred through referral requests on case-to-case basis. The referrals were often timely, yet at times too late, causing i) prolonged non-beneficial ICU stay, ii) DAMA, iii) death while in the ICU.

Within the ICU setting, clinical evaluation of the intensivist along with SOFA – Sequential Organ Failure Assessment (SOFA) score as the standard objective metric, informed prognostication and clinical decision-making.

Proceedings of the Meeting

Need for appropriate criteria

- Participants appreciated the idea of using criteria for timely identification of ICU patients who may require discussions around prognosis and transitions in care.

- There was a general agreement in several of the prognostic indicators that were brought up by different intensivists.
- There were conflicts related to some of the prognostic indicators – especially related to patients with brain injury.
- The prognostic indicators were appraised based on
 - certainty of absence of reversible contributors,
 - reliability across circumstances,
 - scope of unintentionally promoting premature restriction of treatment,
 - and complexity with navigating and aligning the family and their expectations during counselling sessions.
- The team demonstrated noteworthy alliances. Certain members' pointers received more attention and deliberation than others.
- The criteria that was most contested against, was APACHE within 24 hours
- ICU readmissions within a short time frame – was received as an important clinical indicator of poor prognosis

Suggested characteristics of resulting framework

- Recommended the framework be simple, clear, interpretable, and quick to apply during routine patient reviews.
- Emphasized the need for the framework to remain clinically practical and feasible for intensivists working in busy ICU environments.
- Highlighted that the framework should support clinicians without adding additional cognitive or documentation burden.
- Emphasized while structured framework can guide decision-making, the treating intensivist's clinical reasoning overrides all.

Key Observations

- The intensivists showed considerable receptiveness to discussing about enhance care appropriate to patient condition and prognosis. This was demonstrated through active participation in brainstorming of pointers and suggestions about standardising the framework.
- They approached the discussion of clinical parameters through the lens of contextual factors such as;
 - unpredictability of clinical responsiveness,
 - individual differences in the trajectory,
 - pro-life family expectations,
 - and personal discomfort with uncertainty.
- The team suggested rigor in the processes, including piloting and regression analysis to identify the weightage of individual parameters.
- The intensivists struggled with the idea of objective prognostication metrics. there was considerable resistance in response to having a tool-based prognostication.
- The intensivists reported that while having criteria is pivotal, having predefined criteria may not fully encompass the complexities of clinical decision-making. They questioned whether decisions could be based exclusively on such parameters.

- The moderator guided the exploration toward examining clinical indicators independently and working towards a framework to trigger reflections, review-discussions of a particular patient; rather than developing a tool.

Action Items

- Extract and consolidate tentative indicators
- Conduct a follow-up survey to validate criteria

Subsequent Outcome

- Following the meeting, the tentative criteria identified during the discussions were circulated among participants for evaluation using a Likert scale to assess their perceived appropriateness. However, the responses demonstrated a considerable proportion of neutral ratings. In view of this, the survey methodology was revised to a binary response format (Yes/No) to facilitate clearer assessment of consensus regarding the inclusion of specific criteria.

Recognizing the concerns raised regarding the potential use of criteria based prescriptive tool, the criteria are now positioned as a supportive framework. This is intended to guide early clinical considerations, including opportunities for communication, identification of supportive care needs, and appropriate introduction or involvement of palliative care services.

The current framework comprises criteria mapped across different time points during the ICU admission to support timely clinical assessment:

- Pre-morbid criteria
- Within 4–6 hours of admission: 8 criteria identified for evaluation, with consensus pending on 2 criteria.
- Within the first 24 hours of admission: 1 criterion identified, with consensus pending.
- Between 2–4 days of admission: 6 criteria identified, with consensus pending on 1 criterion.

Another focus group discussion to revisit the drafted framework is planned to occur in the month of July.

Recommendations by the moderator

- As the tool is intended to support clinical conversations, continued focus on communication skills may help enhance its acceptability and utility.

ApTCare-ICU– Guiding Framework for Appropriate Transitions in Care for PwSI ¹		
A. Applicable at the IP setting, or within 4-6 hours of ICU admission: Pre-morbid Baseline Appraisal		
1. ☐ Is Pre-morbid Frailty Score ≥ 7 ?	Consensus 80:20	
2. ☐ Charlson Comorbidity Index of ≥ 5 OR an end-stage disease condition		
3. ☐ ≥ 3 non-elective ICU admissions over the last 6 months? OR > 2 non-elective ICU admissions during this hospitalisation		
4. ☐ Is there poor baseline nutrition hypo-albuminaemia or body mass index < 20	Consensus 80:20	
5. CPR prior to ICU admission, with Return of spontaneous circulation (ROSC), but GCS persisting < 8 ; OR objective evidence of severe, irreversible brain injury, with NO known reversibles, despite > 24 hours of ICU care		
6. ☐ Diagnosis of advanced neurodegenerative conditions: Parkinsons, ALS, Parkinson's Disease, Multiple-Sclerosis		
7. ☐ ICU readmission for the same condition < 30 days		
8. ☐ Expressed / documented wish to WH -Life-Support-Treatment, OR admission to ICU from a hospice facility		
If > 2 of the above premorbid conditions exist à Initiate discussions on; - o guarded prognosis, non-escalation to aggressive care - o elicit family goals & manage expectations with the family		
1 st 24 hours ☐ APACHE 2 of ≥ 25	Consensus poor	
B. Applicable within 2-4 days of Care in the ICU: Responsiveness to ICU interventions and Perspectives		
1. ☐ Serially recorded SOFA – Sequential Organ Failure Assessment (SOFA) score on Day 0, Day 3 $\pm$ Day 5 indicating unstable, deteriorating, or decreasing reversibility of patient's condition/s		
2. ☐ Persistent low Glasgow Coma Scale GCS of < 8 ; and, there is objective evidence of severe / irreversible brain injury (e.g. clinical, imaging); or despite 72 hours of relevant ICU-care		
3. ☐ Is there serious intractable persistent infection, unresponsive guideline-based therapy	Consensus 80:20	
4. ☐ 2-3 intensivists $\pm$ specialists concur with non-beneficence of ongoing / escalating ICU interventions in this patient		
5. ☐ The considered answer is NO to the “Surprise Question” - ‘Would I be surprised if this patient were to die during this hospitalization?’		
6. ☐ Is the patient / family requesting for de-escalation, DAMA, Comfort Care / Palliative Care?		
If any of the above conditions exist à Initiate discussions on guarded prognosis; elicit & manage expectations with the family		

¹ Person with Serious illness